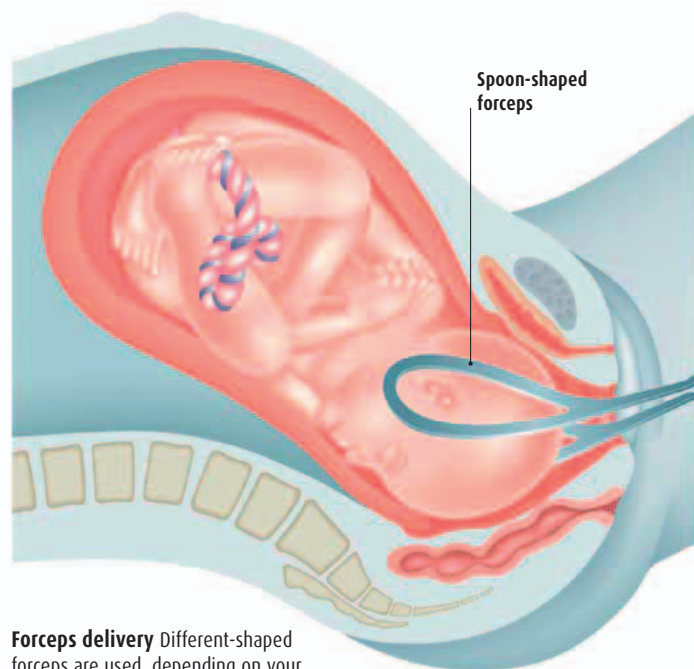


Q What does it mean when a delivery is assisted and how is this done?

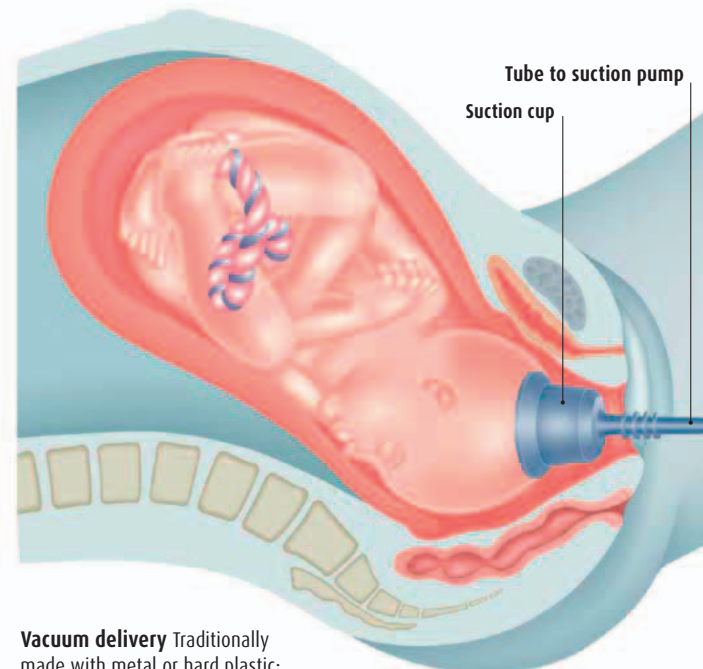
An assisted, or instrumental, delivery is when an obstetrician uses a specially designed instrument, either forceps or a vacuum extractor, to help birth your baby vaginally, avoiding the need for a cesarean section.

Forceps These are tongs, usually made of metal, that open and close like scissors and are curved to fit the shape of your baby's head. The doctor will carefully position the tongs either side of your baby's head, then will gently pull during a contraction to guide your baby out.



Forceps delivery Different-shaped forceps are used, depending on your baby's position and the direction in which she needs to be turned.

Vacuum extractor This is a device with a plastic or metal cup at one end that is placed on your baby's head. A vacuum is created by the suction cup, making it stick to the head, then, during your contraction, the doctor will pull on the cup to help deliver your baby.



Vacuum delivery Traditionally made with metal or hard plastic; some modern cups are silicone, which can be easier to insert.

Q What reasons could there be for me to need an assisted delivery?

An assisted delivery is only done when there is concern for the well-being of either you or your baby in the second stage of labor. For example, if your baby shows signs of distress; if you have a heart condition or high blood pressure that makes the effort of natural birth dangerous for you; if your labor has been protracted and you're exhausted; or if you've had an epidural that has affected the ability of your pelvic muscles to turn your baby so that she can emerge safely. The goal is for you to have a vaginal delivery while minimizing the risks to you and your baby, and the procedure is only done if it is considered the safest option.

Q If I do need an assisted delivery, will they just jump in with the instruments?

Before the procedure takes place, you will be asked for your consent. This can be given verbally if the procedure is being done in the delivery room, or you will need to give written consent if you are moving to an operating room. First, you will need to be in the best position for the procedure to be performed. The doctor will ask you to sit, leaning back slightly, with your knees drawn up, and thighs open. You could be asked to place your feet in footrests, which eases the strain on your back. You will have a catheter inserted to empty your bladder before the procedure can begin; and you may need to

have an episiotomy (see p.217) to make the opening of your vagina wide enough to allow the forceps or vacuum extractor through to wrap around your baby's head. To relieve the discomfort of the procedure, you will be given either a local anesthetic inside the vagina, known as a pudendal block, or sometimes an epidural, which may be recommended if your baby needs help rotating in your pelvis. If you do have an epidural, this makes the transition to a cesarean section easier should the assisted delivery be unsuccessful. Your doctor will explain what is happening and why, and will encourage you to ask as many questions as you need. Your birth partner can play an important role in this situation by asking questions to help make sure you are both fully informed.